

Aetna Member Services SNP Job Aids

Scope closed July 24, 2026 | Two one-page peer quick references plus this audit record

BLUF: The final job aids preserve the operational strength of the photographed Aetna materials - plan-specific verification and use of current member documents - while replacing outdated, overly broad or incomplete explanations with current federal definitions and 2026 Aetna public materials.

1. Materials Reviewed

The source set included photographed Aetna Better Health Center of Excellence Member Services materials, Medicare and Medicaid foundation pages, a Benefits Information call script revised February 23, 2025, and member/authorized-representative and verification-process materials. The photographs were used as operational context; public legal and plan definitions were independently revalidated.

The key operational principle retained from the Aetna script is that representatives should use the current plan-specific member handbook or comparable controlling plan source when explaining benefits.

2. Publication Standard

- **Authority:** Primary sources first - current eCFR, CMS program pages/guidance, then Aetna 2026 public plan materials.
- **Scope labeling:** Federal definition, Aetna public information, state/plan-specific rule, internal workflow or peer-created call language.
- **No benefit promises:** No statement implies that an SNP acronym guarantees a particular allowance, copay, network rule or Medicaid responsibility.
- **Currency:** eCFR Title 42 was current through July 23, 2026; Aetna SNP pages reviewed were updated in 2026.
- **Usability:** Each aid must be readable during a live call without page flipping.

3. Statement-by-Statement Validation Matrix

JOB-AID STATEMENT	PRIMARY SUPPORT	DECISION / LIMIT
SNPs are MA coordinated-care plans for defined special-needs populations.	CMS Special Needs Plans; 42 CFR 422.2.	Published. Describes the category without implying uniform benefits.
D-SNP = Medicare-eligible person receiving Medicaid assistance; coordinates both programs.	42 CFR 422.2 definition of D-SNP.	Published. Added state/plan verification because Medicaid responsibility varies.
C-SNP enrollment is limited to qualifying severe/disabling chronic conditions.	42 CFR 422.2; CMS C-SNP page; Aetna C-SNP page updated May 8, 2026.	Published. Did not list every possible CMS condition; used plan-specific verification language.
I-SNP includes FI-SNP, IE-SNP and HI-SNP subtypes.	42 CFR 422.2 current definitions.	Published. Corrects the common oversimplification that I-SNP only means nursing-home residence.
Institutionalized generally requires 90+ days in a qualifying setting.	42 CFR 422.2; Aetna I-SNP page updated April 13, 2026.	Published with "generally" because the federal definition includes specified settings and CMS-approved equivalents.
FIDE requires a single entity, broad Medicaid benefit coverage, integrated processes, EAE and whole service area for 2025+.	42 CFR 422.2 FIDE definition.	Published in compressed form. Individual coverage remains subject to the state contract and approved carve-outs.
HIDE covers LTSS or behavioral health under a capitated Medicaid contract and whole service area for 2025+.	42 CFR 422.2 HIDE definition; 42 CFR 422.107 carve-outs.	Published. Explicitly states HIDE is not simply "FIDE-lite" and does not alone prove EAE.
Coordination-only D-SNP is not FIDE/HIDE but still coordinates and has state-contract duties.	CMS Medicare Managed Care Manual Ch. 16-B; 42 CFR 422.107(d).	Published. Avoided implying the D-SNP directly pays every Medicaid service.
AIP is not synonymous with FIDE or HIDE.	42 CFR 422.561; CMS integrated appeals/grievance rules.	Published as a compact distinction because AIP describes an integrated D-SNP/Medicaid arrangement and process requirements.
2026 Integrated Care SEP is monthly only to create aligned enrollment.	CMS CY 2026 MA/Part D Enrollment and Disenrollment Guidance, section 30.6.35.	Published as a reminder, not an enrollment script. Current internal enrollment workflow still controls.
Quote benefits from current plan-specific sources.	Photographed Aetna COE Benefits Information call script, revised 02/23/2025; current Aetna webpages repeatedly direct users to plan-specific availability and documents.	Published as the controlling call-handling principle.

4. Red-Team Changes and Exclusions

Older COE plan list: The 2025 photographed script lists MMP and state products that cannot be treated as a complete 2026 national Aetna D-SNP map. The final aids do not reproduce that list.

Generic benefit language: Food, utilities, transportation, OTC, dental, vision, hearing and other supplemental benefits were not presented as universal SNP benefits. Aetna states that availability and eligibility vary by plan and benefit.

Medicaid income example: The photographed 2026 poverty-guideline example was excluded. A federal poverty guideline is not a universal Medicaid eligibility threshold; eligibility varies by state and category.

Mandatory/optional Medicaid benefits list: The list was used to reinforce state variation but excluded from the one-page aids because it is too detailed and would distract from the SNP decision task.

HIPAA verification procedure: The photographed internal verification workflow was kept outside the SNP pages. Internal identifier counts and authorized-representative procedures must come from the current controlled Aetna desktop, not a public federal definition.

Prior authorization examples and call-center hours: Excluded because they are plan-, state-, service- and time-specific and may change independently of SNP definitions.

Aetna branding: No Aetna logo or claim of formal approval appears. The documents are clearly labeled peer-created quick references.

5. Facts, Interpretations, Assumptions and Open Questions

Facts

- Federal definitions in 42 CFR 422.2 distinguish D-SNP, C-SNP and I-SNP, and separately define FIDE and HIDE.
- FIDE SNPs require exclusively aligned enrollment for plan year 2025 and later.
- HIDE SNP status is based on specified Medicaid capitated coverage; it does not by itself establish exclusively aligned enrollment.
- Aetna's 2026 public pages state that SNP availability, conditions and benefits vary by location and plan.

Interpretations

- For Member Services, the most useful distinction is not merely "more" versus "less" integrated, but which entity owns the requested benefit and which plan document controls.
- The safest member-facing language explains coordination while reserving the exact benefit determination for the member's plan and state.

Assumptions

- The job aids are intended as peer training/reference documents, not controlled Aetna policy documents.
- Coworkers will have access to current internal systems, member handbooks and plan documents during calls.

Open Questions for Internal Approval

- Which internal desktop or knowledge article should be named as the controlling source for each plan line?
- Should Aetna compliance/training approve the peer reference before broad distribution?
- Should an internal-only version include system names, transfer queues or state-specific workflows?

6. Primary Source List

42 CFR 422.2 - Definitions

<https://www.ecfr.gov/current/title-42/part-422/section-422.2>

42 CFR 422.107 - Requirements for D-SNPs

<https://www.ecfr.gov/current/title-42/part-422/section-422.107>

42 CFR 422.561 - Applicable integrated plan and related definitions

<https://www.ecfr.gov/current/title-42/part-422/section-422.561>

CMS - Special Needs Plans

<https://www.cms.gov/medicare/enrollment-renewal/special-needs-plans>

CMS - Chronic Condition Special Needs Plans

<https://www.cms.gov/medicare/enrollment-renewal/special-needs-plans/chronic-conditions>

CMS - D-SNP Integration and CY 2026 Integrated D-SNP resources

<https://www.cms.gov/medicare/medicaid-coordination/about/dsnps>

CMS - CY 2026 MA and Part D Enrollment/Disenrollment Guidance

<https://www.cms.gov/files/document/cy-2026-cd-enrollment-and-disenrollment-guidance.pdf>

CMS - Medicare Managed Care Manual, Chapter 16-B

<https://www.cms.gov/files/document/r131mcm.pdf>

Aetna Medicare - Special Needs Plans

<https://www.aetna.com/medicare/compare-plans-enroll/snp-special-needs-plans.html>

Aetna Medicare - D-SNP

<https://www.aetna.com/medicare/compare-plans-enroll/medicare-advantage-dsnp-plans.html>

Aetna Medicare - C-SNP

<https://www.aetna.com/medicare/compare-plans-enroll/csnp-chronic-condition-special-needs-plans.html>

Aetna Medicare - I-SNP

<https://www.aetna.com/medicare/compare-plans-enroll/isnp-institutional-special-needs-plans.html>

Aetna Better Health Illinois - 2026 FIDE materials

<https://www.aetnabetterhealth.com/illinois/providers/fide>

Aetna Better Health Michigan - 2026 HIDE Provider Orientation

https://www.aetnabetterhealth.com/content/dam/aetna/medicaid/michigan/provider/pdf/abhmi_Aetna_HIDE_2026.pdf

Final status: The two one-page job aids passed source review and were designed as fast peer references. They are not substitutes for current plan documents, controlled Aetna procedures, or compliance approval.